

# SPARTAN COACHING

DISCIPLINE | EMPATHY | STRATEGY

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## DIFFICULT CONVERSATION GUIDE

### Frameworks for Sensitive End-of-Life Conversations

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Complete guides for five high-stakes scenarios: family reluctance, patient refusal, physician resistance, competitive displacement, and census loss conversations.

## Difficult Conversation Guide

Presence, Patience, and Prepared Language for Hard Moments

### THE FOUNDATION: POSTURE BEFORE WORDS

Difficult conversations in hospice are not won with clever scripts. They are won with presence. The person across from you — whether it is a grieving family member, a defensive physician, or a frustrated discharge planner — does not primarily need to hear the right words. They need to experience being genuinely heard by someone who is not trying to rush through the conversation to get to their agenda.

The three posture principles for every difficult conversation are: slow down, listen longer than is comfortable, and resist the urge to solve before the other person is ready to be helped. These principles do not come naturally under pressure. Practice them in low-stakes conversations so they are available to you in high-stakes ones.

### UNIVERSAL PREPARATION CHECKLIST

- ☐ Review everything you know about the situation and the person: their role, their history, any previous interactions
- ☐ Identify their most likely concern: what do they need from this conversation to feel like it was worthwhile?
- ☐ Prepare 2 to 3 open-ended questions rather than a script or talking points
- ☐ Check your own emotional state: are you calm enough to be fully present without an agenda driving your energy?
- ☐ Identify what a successful outcome looks like for them, not just for you

### SCENARIO 1: FAMILY IS NOT READY TO ACCEPT HOSPICE

#### What Is Happening Beneath the Surface

"Not ready" almost always means one of three things: they do not yet believe the prognosis is what the doctors are saying, they feel that choosing hospice is giving up on their loved one, or they carry unresolved guilt about past caregiving that makes this decision feel like the final evidence of failure. Your job is to find out which one it is before saying anything else.

#### Opening the Conversation

*"I want to be upfront with you: I am not here to convince you of anything or to rush you toward any decision. What I would really like to understand is what is on your mind right now. What feels like the hardest part of what you are facing?"*

## When They Say "We Are Holding Out Hope"

*"I hear that, and I think hope is exactly the right thing to hold onto. What I have seen is that choosing hospice does not mean giving up hope. It means choosing to focus your hope on the things that matter most to your [family member] right now: comfort, dignity, time with the people they love. Can I share what that looks like in practice?"*

## When They Say "We Do Not Want to Give Up"

*"That response tells me a lot about who you are and what your [family member] means to you. Hospice actually does not require you to give up. Your [family member] can still receive treatment for any condition unrelated to their terminal diagnosis. And they continue to have their own physician. What changes is that a team of specialists steps in to manage their comfort and support your family through what comes next. Would it be helpful to talk about what that actually looks like day to day?"*

## When They Are Silent or Crying

Stop talking. Wait. Let the silence exist without filling it. The most powerful thing you can do when someone is crying is to be present and quiet. Place a hand on the table if appropriate. After they have had time, say simply:

*"Take all the time you need. There is no rush here."*

## SCENARIO 2: PATIENT REFUSES TO DISCUSS HOSPICE

### What Is Happening

Patients who refuse to discuss hospice are almost always processing a fear underneath the refusal. The most common fears: fear that accepting hospice accelerates death, fear of losing control over their own healthcare decisions, and fear that their doctor or family has given up on them. Naming these fears gently opens the conversation that lecturing closes.

### Opening Without Pressure

*"I am not here to talk you into anything. I just wondered if you would be willing to share with me what specifically worries you about hospice. I have heard a lot of misconceptions over the years and I want to make sure you have accurate information to make whatever decision is right for you."*

## Addressing the "Hospice Speeds Up Death" Misconception

*"I understand why people believe that, and it is one of the most common concerns I hear. The research actually shows the opposite: patients who enroll in hospice with serious illnesses like heart failure and cancer have statistically similar or in some cases longer survival compared to patients who continue aggressive treatment, with significantly better symptom control and quality of life. Hospice does not shorten life. It focuses on how that life is lived."*

## SCENARIO 3: PHYSICIAN DISMISSES THE HOSPICE CONVERSATION

### What Is Happening

Physician resistance to hospice is almost always rooted in one of three things: they believe referring to hospice signals clinical failure, they have had a bad experience with a hospice organization in the past, or they are not yet convinced the patient meets eligibility criteria. Each requires a different approach.

### When the Physician Feels Hospice Signals Failure

*"I hear that concern a lot from physicians I work with, and I think it reflects how deeply you are invested in your patients. What I have seen is that the physicians whose patients do best in hospice are the ones who stay involved. You do not give up your patient when you refer to hospice. You gain a team of specialists who support your care plan. Many physicians describe it as the most effective handoff they have made."*

### When the Physician Had a Bad Prior Experience

*"I appreciate you telling me that. Can I ask what happened? I want to understand the specific gap so I can tell you honestly whether our team does things differently, and if we do not, I am not going to try to paper over that with promises."*

If the issue was communication, explain specifically how you handle clinical updates. If it was response time, explain your after-hours coverage protocol. If it was a clinical decision they disagreed with, acknowledge it and ask whether they would be open to a single patient trial where they stay closely involved in the care decisions.

### When the Physician Questions Eligibility

*"I would never want you to refer a patient who does not meet eligibility criteria — that creates regulatory risk for you and your practice and it is not something we would support. What I can offer is an informal clinical consultation with our medical director to look at the specific patient together. That way, the eligibility determination is clinical, not sales-driven."*

## SCENARIO 4: A REFERRAL SOURCE IS USING A COMPETITOR

### What Is Happening

Competitor displacement requires patience and a long-term perspective. The worst thing you can do is speak negatively about the competitor, pressure the referral source to switch, or make promises you cannot keep. The most effective strategy is to become so consistently valuable that the relationship you build makes the switch feel natural over time.

### Opening Without Attacking

*"I have a lot of respect for the relationship you have built with [competitor]. I am not here to disrupt that. What I am interested in is whether there are specific situations — particular patients, diagnoses, or family circumstances — where having a second option would be useful. I would rather start with the edge cases and earn the right to be considered more broadly over time."*

### When They Mention Dissatisfaction with the Competitor

Listen completely before saying anything. Do not express satisfaction that they are unhappy. Do not immediately offer yourself as the solution. After they have finished:

*"I am sorry to hear that. That is not the experience patients and families deserve. What would need to be different for you to feel confident about trying another provider?"*

## SCENARIO 5: YOU HAVE A SERVICE QUALITY PROBLEM TO ADDRESS

### What Is Happening

When your hospice has let a referral source down — a missed visit, a late response, a communication breakdown — the fastest way to lose the relationship is to minimize the problem or make excuses. The fastest way to strengthen the relationship is to own the failure completely, explain what you know about what happened, and demonstrate what has changed.

### Addressing the Issue Directly

*"I need to talk to you about what happened with [situation]. I want to start by saying I am sorry. What you experienced was not acceptable and it does not reflect the standard we hold ourselves to. Can you tell me exactly what happened from your perspective so I make sure I have the complete picture?"*

After they have shared the full account:

*"Thank you for telling me all of that. Here is what I know about what happened from our end: [specific explanation without excuses]. Here is what has changed or what I am committing to personally going forward: [specific, measurable commitment]. And I want to ask: is there anything else I should know, or anything you need from me right now?"*

## The Follow-Through Is Everything

Whatever you commit to in this conversation, deliver it faster than you promised and with more specificity than they expected. Service recovery is one of the most powerful trust-building experiences in any relationship, but only if the follow-through matches or exceeds the expectation you set in the moment of apology. If you commit to a callback by Thursday, call Wednesday. If you commit to providing a clinical update weekly, provide it every Tuesday at the same time.

## LANGUAGE TO USE AND LANGUAGE TO AVOID IN DIFFICULT CONVERSATIONS

### Use Language That Invites and Opens

- "Tell me more about what is behind that concern."
- "Help me understand what that situation looks like from your perspective."
- "What would need to be true for this to feel like the right path?"
- "What matters most to you and your [patient / resident / family member] right now?"
- "Is there anything about what I just shared that does not feel right to you?"

### Avoid Language That Closes or Pressures

- "You should really consider hospice for this patient." — Too directive, removes their agency
- "Most families in your situation..." — Generalizes their specific experience, feels dismissive
- "If I were you, I would..." — Makes the conversation about you and presumes equivalence
- "They definitely qualify for hospice." — Clinical determination belongs to the clinical team, not sales
- "Our hospice is the best option here." — Comparative claims create distrust, not confidence
- "I understand completely." — Unless you have been in their exact situation, this feels hollow

## THE CORE OF DIFFICULT CONVERSATIONS

The most skilled hospice sales professionals are not the most persuasive. They are the most patient and the most genuinely curious. In a field where you are regularly encountering grief, fear, clinical complexity, and institutional pressure, the people who stay grounded, listen without an agenda, and speak from a place of genuine care for patient outcomes are the ones who build relationships that last for careers. That is the Spartan standard. Master the fundamentals of presence before mastering the art of persuasion.